Recreational Space Accessibility and Health Outcomes Across Victorian LGAs

ETC5543 Research Project

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30 October 2026

Abstract

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1 Introduction

1.1 Background and Motivation

Access to parks and recreational spaces has become an important consideration in Melbourne's urban planning. In October 2024, ABC News reported on a proposal by Infrastructure Victoria to open more public school grounds for community use outside school hours. The proposal was estimated to improve access to nearby recreational spaces for approximately 450,000 Melbourne residents (Jonscher 2024). This initiative reflects efforts to make better use of existing public spaces and improve recreational opportunities for local communities.

Parks, natural areas, and sport and recreation facilities provide spaces for physical activity, outdoor recreation, and social interaction. These activities can contribute to people's physical and mental wellbeing. However, recreational opportunities may vary across communities depending on their local environments and the types of facilities available.

Geographical accessibility is an important aspect of recreational opportunities. It concerns the spatial relationship between where people live and the recreational spaces they may wish to use. Previous research has used distance to the nearest park as one way of measuring accessibility (Shoari et al. 2020). This approach provides a way to examine how proximity to recreational spaces differs between residential locations.

However, greater geographical accessibility to recreational spaces does not necessarily mean higher levels of physical activity or better health outcomes. Factors such as population density and socioeconomic disadvantage may be associated with both recreational accessibility and health outcomes. Examining these relationships while considering such factors is therefore important for understanding the associations between recreational accessibility and population health.

1.2 Research Question and Objectives

The primary research question of this study is:

Is greater accessibility to recreational spaces associated with higher levels of physical activity and better health outcomes across Victorian Local Government Areas (LGAs)?

To address this question, the study has four main objectives:

1. **Measure recreational accessibility:** Develop LGA-level indicators of geographical accessibility to different types of recreational spaces, including parks, natural areas, and sport and recreation

facilities.

2. **Explore geographical patterns:** Examine variations in recreational accessibility, physical activity, and health outcomes across Victorian LGAs.
3. **Investigate statistical associations:** Assess the relationships between recreational accessibility and physical activity and health outcomes, including obesity, psychological distress, and poor self-rated health.
4. **Assess additional explanatory value:** Determine whether recreational accessibility explains additional variation in physical activity and health outcomes after accounting for socioeconomic disadvantage and population density.

1.3 Significance and Contributions

This study provides an LGA-level perspective on recreational space accessibility and its relationship with physical activity and health outcomes across Victoria. By examining different types of recreational spaces, the study offers a broader understanding of how geographical accessibility varies across communities and how these differences relate to population health.

A key contribution of this project is the integration of recreational space, population, socioeconomic, and health data to construct a consistent dataset for analysis across Victorian LGAs. Using population-based spatial locations, the study develops accessibility measures that reflect the geographical proximity of residents to different recreational spaces. This provides a more population-focused perspective on accessibility across LGAs.

The study also contributes empirical evidence on the associations between recreational accessibility and multiple health outcomes, while considering socioeconomic disadvantage and population density. These findings may provide useful insights for future research and discussions about recreational space planning and public health, while recognising that the observed associations do not establish causal relationships.

2 Data and Preparation

2.1 Study Area and Unit of Analysis

This study focuses on Victoria, Australia, using LGAs as the geographical unit of analysis. The LGA level was selected because the physical activity and health outcome indicators are available at this geographical scale, allowing recreational accessibility and other explanatory variables to be examined alongside these outcomes.

As the datasets used in this study have different geographical structures and levels of aggregation, spatial data processing was required to establish a consistent LGA-level framework. The final analytical dataset includes 79 Victorian LGAs, with each LGA representing one observation.

2.2 Data Sources

This study integrates spatial, recreational facility, population, socioeconomic, and health data from multiple sources. These datasets provide information on the distribution of recreational spaces, population characteristics, and health outcomes across Victorian LGAs. The following sections describe the main datasets used in the analysis.

2.2.1 LGA Boundaries

LGA boundary data were obtained from the *Vicmap Admin – Local Government Area (LGA) Polygon Aligned to Property* dataset, provided by the Victorian Government (Department of Transport and Planning 2026a). The dataset contains geographical boundaries of LGAs across Victoria.

These boundaries were used as the common geographical framework for the study, allowing datasets with different spatial structures and levels of aggregation to be integrated at the LGA level.

2.2.2 Recreational Parks and Natural Areas

Spatial data on recreational parks and natural areas were obtained from the Vicmap Planning – Planning Scheme Zone Polygon dataset, provided by the Victorian Government's Department of Transport and Planning (Department of Transport and Planning 2026b). The dataset contains geographical boundaries of planning zones across Victoria.

Two planning zone categories were selected: Public Park and Recreation Zone (PPRZ) and Public Conservation and Resource Zone (PCRZ). PPRZ was used to represent recreational parks, while PCRZ was

used to represent natural areas. These spatial boundaries provide the basis for measuring geographical accessibility to parks and natural areas, as described in Section 3.

2.2.3 Sport and Recreation Facilities

Data on sport and recreation facilities were obtained from the *Sport and Recreational Facilities List*, published by the Victorian Government (Department of Jobs, Skills, Industry and Regions 2024). The dataset contains information on individual recreational facilities across Victoria, including their geographical coordinates, facility types, and sports played.

These data were used to identify the geographical locations of sport and recreation facilities, including indoor facilities. The facility locations provide the basis for measuring geographical accessibility to these recreational spaces, as described in Section 3.

2.2.4 Physical Activity and Health Outcomes

Data on physical activity and health outcomes were obtained from the Victorian Population Health Survey (VPHS) 2023, published by the Victorian Agency for Health Information (Victorian Agency for Health Information 2023). The survey provides population health estimates at the LGA level.

Five indicators were selected for this study: physical inactivity, physical activity (≥150 minutes per week), obesity (BMI ≥30), high or very high psychological distress (K10), and fair or poor self-rated health. These indicators cover physical activity, physical health, and mental wellbeing.

Age-adjusted percentages for the total population (People) were used to support comparisons across LGAs. These indicators serve as the outcome variables for examining their associations with recreational accessibility.

2.2.5 Population and Socioeconomic Data

Population data were obtained from the Australian Bureau of Statistics (ABS). The 2021 Mesh Block boundaries (Australian Bureau of Statistics 2021) and Census Mesh Block Counts (Australian Bureau of Statistics 2022) were used to represent the spatial distribution of residents across Victoria. These data provided the residential locations and population weights used in the accessibility analysis.

LGA-level population estimates for 2023 were obtained from the ABS Regional Population dataset (Australian Bureau of Statistics 2024). These data provided information on population size and density across Victorian LGAs. Population density was included as a control variable in the regression analysis.

Socioeconomic data were obtained from the ABS Socio-Economic Indexes for Areas (SEIFA) 2021 dataset (Australian Bureau of Statistics 2023). The Index of Relative Socio-economic Advantage and Disadvantage (IRSAD) was selected to represent socioeconomic conditions at the LGA level. Higher IRSAD scores indicate relatively greater socioeconomic advantage, while lower scores indicate relatively greater socioeconomic disadvantage. IRSAD was also included as a control variable in the regression analysis.

3 Measuring Recreational Accessibility

4 Statistical Methods

4.1 Outcomes and Explanatory Variables

The analysis examined five outcomes across Victorian Local Government Areas (LGAs), including less than moderate physical activity (inactivity), meeting the recommendation of at least 150 minutes of moderate-to-vigorous physical activity (MVPA) per week, obesity, psychological distress, and fair or poor self-rated health. Each outcome was measured as a percentage at the LGA level.

The explanatory variables comprised socioeconomic conditions, population density, and six recreational accessibility indicators. Socioeconomic conditions were measured using the Socio-Economic Indexes for Areas (SEIFA) Index of Relative Socio-economic Advantage and Disadvantage (IRSAD). Population density was included to account for differences in settlement patterns across LGAs. The accessibility indicators included median distance to parks, natural areas, recreational facilities, and indoor facilities, the percentage of facilities that were indoor facilities, and the number of sport types available.

The analysis assessed whether these accessibility indicators provided additional explanatory power beyond socioeconomic conditions and population density.

4.2 Predictor Transformations

Predictor transformations were applied to reduce skewness. Population density and the four distance measures were log-transformed before standardisation. The percentage of indoor facilities was square-root transformed, while SEIFA IRSAD and sport-type count were retained on their original scales before standardisation. All predictors were standardised using z-scores, calculated by subtracting the sample mean and dividing by the sample standard deviation. The indoor-facility share and sport-type count were reverse-coded so that higher values represented a lower indoor-facility share and fewer sport types. SEIFA IRSAD was not reverse-coded, meaning that higher values continued to indicate greater socioeconomic

advantage. A park–facility composite was constructed as the arithmetic mean of the standardised park-distance and facility-distance indicators. This composite was used in Model A to represent these two dimensions jointly.

4.3 Hierarchical Model Specification

Ordinary least squares (OLS) regression models were fitted separately for each of the five outcomes. A sequence of nested and extended models was used to evaluate the explanatory contribution of socioeconomic conditions, population density, and recreational accessibility.

Table 1: Specification of the hierarchical OLS regression models evaluating socioeconomic conditions, population density, and recreational accessibility.

Model	Predictors	Purpose
M0	SEIFA	Establish the socioeconomic baseline
M1	SEIFA + log-transformed population density	Assess the additional contribution of population density
M2	M1 + one accessibility indicator	Evaluate each accessibility indicator individually
A	M1 + park–facility composite + natural-area distance + indoor-facility distance	Evaluate a compact combination of accessibility indicators
B	M1 + park distance + facility distance + natural-area distance + indoor-facility distance	Evaluate the four distance indicators jointly
C	M1 + all six accessibility indicators	Evaluate the full set of accessibility indicators jointly

For each outcome, M0 and M1 were compared to assess whether population density provided additional explanatory power beyond SEIFA IRSAD. Each M2 model added one accessibility indicator to M1, allowing its individual contribution to be assessed. Models A, B and C examined the joint contribution of multiple accessibility indicators using alternative predictor combinations. The extended models were compared with M1 to evaluate whether recreational accessibility improved explanatory power after accounting for socioeconomic conditions and population density.

4.4 Model Comparison and Statistical Inference

Model performance was primarily evaluated using adjusted R^2, which measures the proportion of variation explained by a model while accounting for the number of predictors. For each extended model, the change in adjusted R^2 was calculated relative to M1:

$$R^2_{\text{adj,extended}} - R^2_{\text{adj,M1}}$$

A positive change indicates an increase in adjusted explanatory power, whereas a negative change indicates that the extended model did not improve adjusted (R^2) after accounting for model complexity.

Statistical evidence for incremental contributions was assessed using HC3 heteroskedasticity-robust standard errors and robust F-tests. For M1, the test evaluated the contribution of population density beyond SEIFA IRSAD. For each M2 model, it evaluated the individual accessibility indicator added to M1. For Models A, B and C, joint F-tests assessed whether the accessibility predictors added to M1 were jointly different from zero.

Adjusted (R^2) and HC3 robust tests were interpreted as complementary measures. Adjusted R^2 was used to compare explanatory power, while the robust tests assessed statistical evidence for the additional predictors. Statistical significance was evaluated at the 5% level. These analyses were conducted at the LGA level and were intended to assess associations and explanatory power rather than establish causal effects.

5 Results

5.1 Baseline Model Performance: M0 vs M1

Model M0 used SEIFA as the sole predictor, while Model M1 additionally included population density. The comparison assessed the explanatory power of socioeconomic conditions and whether population density provided additional explanatory power beyond socioeconomic conditions.

Table 2: Baseline model performance across five outcomes. *Note: Δ Adjusted R^2 is calculated as the adjusted R^2 of M1 minus that of M0. HC3 p-values test the additional contribution of population density after controlling for SEIFA.*

Outcome	M0 Adjusted R^2	M1 Adjusted R^2	Δ Adjusted R^2	HC3 p-value
Less than moderate activity	0.617	0.612	-0.005	0.795
At least 150 min/week MVPA	0.163	0.161	-0.002	0.425
Obesity	0.626	0.645	+0.019	0.121
Psychological distress	0.272	0.317	+0.046	0.029
Fair/poor self-rated health	0.437	0.465	+0.027	0.147

The explanatory power of SEIFA varied substantially across outcomes. Under M0, adjusted R^2 ranged from 0.163 for MVPA to 0.626 for obesity, indicating that socioeconomic conditions explained different proportions of the variation across health and physical activity outcomes.

Adding population density produced mixed results. As shown in Table 2,adjusted R^2 increased for obesity, psychological distress and fair/poor self-rated health, with the largest increase observed for psychological distress (+0.046). However, only psychological distress showed statistically significant evidence of an additional contribution from population density at the 5% level (HC3 p = 0.029). The increases for obesity and fair/poor self-rated health were not statistically significant. For inactivity and MVPA, adjusted R^2 decreased slightly after population density was added.

Overall, population density provided additional explanatory power for psychological distress, but its contribution was not consistently supported across the five outcomes. These results establish M1 as the baseline for evaluating whether recreational accessibility provides further explanatory power.

5.2 Individual Accessibility Contributions: M1 to M2

To examine the individual contribution of recreational accessibility, six M2 models were fitted for each outcome, with each model adding one accessibility indicator to M1. Changes in adjusted R^2 and HC3

robust tests were used to evaluate the incremental explanatory contribution of each indicator.

Table 3: Strongest individual accessibility contributions by outcome. *Note: For each outcome, the indicator shown is the one associated with the highest adjusted R^2 among the six individual M2 models. Δ Adjusted R^2 is calculated relative to M1. The complete results for all 30 M2 models are provided in Appendix [1].*

Outcome	Indicator added	M2 Adjusted R^2	Δ Adjusted R^2	HC3 p-value
Less than moderate activity	Natural-area distance	0.680	+0.068	0.001
At least 150 min/week MVPA	Indoor share (reversed)	0.206	+0.045	0.357
Obesity	Natural-area distance	0.652	+0.008	0.121
Psychological distress	Indoor share (reversed)	0.345	+0.028	0.125
Fair/poor self-rated health	Natural-area distance	0.470	+0.005	0.165

The contribution of individual accessibility indicators differed across outcomes. The clearest improvement was observed for inactivity. Adding natural-area distance increased adjusted R^2 from 0.612 under M1 to 0.680, with statistically significant evidence of an additional contribution (HC3 p = 0.001). The reversed indoor-facility share also improved the adjusted R^2 for inactivity to 0.662 (Δ Adjusted R^2 = 0.050; HC3 p = 0.039). Therefore, both indicators provided evidence of additional explanatory power for inactivity when considered individually. For the other four outcomes, the indicators associated with the highest adjusted R^2 produced smaller improvements, and none had a statistically significant HC3 p-value at the 5% level. For MVPA, the reversed indoor-facility share yielded the largest increase in adjusted R^2 (+0.045), but the evidence was not statistically significant (p = 0.357). Similarly, the largest individual improvements for obesity, psychological distress, and fair/poor self-rated health were not statistically significant.

These findings suggest that individual accessibility indicators provided more substantial additional explanatory power for inactivity than for the other outcomes. However, the results varied across outcomes,

and the strongest-performing indicator differed between them. The next section therefore examines whether combining accessibility indicators provides further explanatory power beyond M1.

5.3 Joint Accessibility Contributions: Models A, B and C

To examine whether recreational accessibility indicators provided additional explanatory power when considered jointly, Models A, B and C were compared with the baseline model M1. Each model included SEIFA and population density, while the accessibility indicators were added in different combinations.

Table 4: Joint contribution of accessibility indicators: Models A, B and C versus M1 *Note: Δ Adjusted R^2 is calculated relative to M1. HC3 joint p-values test whether the accessibility predictors added to each model are jointly different from zero. Values reported as <0.001 are based on the unrounded p-values.*

Outcome	Model	Adjusted R^2	Δ Adjusted R^2	HC3 joint p-value
Less than moderate activity	M1	0.612	–	–
	A	0.713	+0.100	<0.001
	B	0.709	+0.097	<0.001
	C	0.712	+0.100	<0.001
At least 150 min/week MVPA	M1	0.161	–	–
	A	0.158	-0.003	0.795
	B	0.155	-0.005	0.666
	C	0.180	+0.020	0.798
Obesity	M1	0.645	–	–
	A	0.644	-0.001	0.479
	B	0.661	+0.016	0.195
	C	0.653	+0.009	0.367
Psychological distress	M1	0.317	–	–
	A	0.336	+0.018	0.347
	B	0.328	+0.010	0.536
	C	0.336	+0.019	0.370

Outcome	Model	Adjusted R^2	Δ Adjusted R^2	HC3 joint p-value
Fair/poor self-rated health	M1	0.465	–	–
	A	0.461	-0.004	0.281
	B	0.454	-0.011	0.411
	C	0.459	-0.005	0.337

The results showed a clear improvement for less than moderate physical activity. All three combined models increased adjusted R^2 from 0.612 to approximately 0.71. The HC3 joint tests were statistically significant for all three models ($p < 0.001$), indicating that the accessibility indicators provided additional explanatory information beyond socioeconomic conditions and population density. For the other four outcomes, the improvements were smaller and were not statistically significant. Model C produced the highest adjusted R^2 for MVPA (0.180), while Model B performed best for obesity (0.661). For psychological distress, Models A and C achieved similar adjusted R^2 values of 0.336. However, none of these models showed a significant joint contribution from accessibility indicators. For fair or poor self-rated health, all three combined models had lower adjusted R^2 values than M1.

To further compare the three combinations, nested model comparisons were conducted between Models A and B, and between Models B and C.

Table 5: Nested comparisons among combined models. *Note: Δ Adjusted R^2 represents the change in adjusted R^2 when moving from the first model to the second. HC3 p-values assess the additional contribution of the predictors introduced in the larger model.*

Outcome	Comparison	Δ Adjusted R^2	HC3 p-value
Less than moderate activity	A → B	-0.003	0.675
	B → C	+0.003	0.567
At least 150 min/week MVPA	A → B	-0.002	0.384
	B → C	+0.025	0.607
Obesity	A → B	+0.017	0.071
	B → C	-0.008	0.797
Psychological distress	A → B	-0.008	0.701
	B → C	+0.008	0.216
Fair/poor self-rated health	A → B	-0.007	0.864

Outcome	Comparison	Δ Adjusted R^2	HC3 p-value
	B → C	+0.006	0.222

The nested comparisons did not show statistically significant improvements when moving from Model A to B or from Model B to C at the 5% significance level. Although adding the separate park and facility distance indicators in Model B increased adjusted R^2 for obesity by 0.017 compared with Model A, this improvement was not statistically significant ($p = 0.071$). Similarly, adding the remaining accessibility indicators in Model C did not significantly improve any outcome.

Overall, the combined results suggest that the explanatory contribution of recreational accessibility varied across health outcomes. The strongest and most consistent evidence was found for inactivity, while the results for the other outcomes provided limited statistical evidence of additional explanatory power.

6 Comparison of Accessibility Dimensions

The individual and combined model results provide further insight into which accessibility dimensions were most relevant to each outcome. Among the individual M2 models, natural-area distance and the reversed indoor-facility share showed the largest improvements for inactivity. Natural-area distance also produced the highest adjusted R^2 among individual accessibility models for obesity and fair or poor self-rated health. For MVPA and psychological distress, the reversed indoor-facility share produced the largest individual improvements. However, the combined model results showed that the contribution of accessibility indicators differed across outcomes. Model A performed best for inactivity and psychological distress, Model C performed best for MVPA, and Model B performed best for obesity. No combined accessibility model improved the adjusted R^2 for fair or poor self-rated health.

Table 6: Summary of the strongest individual and combined accessibility models.

Outcome	Strongest individual indicator (M2)	Best combined model	Interpretation
Less than moderate activity	Natural-area distance	Model A	Clear improvement from accessibility indicators, supported by significant individual and joint tests

Outcome	Strongest individual indicator (M2)	Best combined model	Interpretation
At least 150 min/week MVPA	Indoor-facility share (reversed)	Model C	Adjusted R^2 increased slightly, but the joint contribution was not significant
Obesity	Natural-area distance	Model B	Some improvement in adjusted R^2, but no significant joint contribution
Psychological distress	Indoor-facility share (reversed)	Model A	Small improvement in adjusted R^2, without significant statistical evidence
Fair/poor self-rated health	Natural-area distance	M1	Accessibility indicators did not improve the adjusted R^2

These findings suggest that no single accessibility dimension consistently provided the greatest explanatory contribution across all health outcomes. Natural-area distance was particularly relevant in the individual models for inactivity, obesity and self-rated health, while indoor-facility share showed the largest individual improvements for MVPA and psychological distress. Nevertheless, these patterns should be interpreted alongside the joint model tests. Therefore, only inactivity showed clear statistical evidence that accessibility indicators collectively added explanatory power.

Overall, recreational accessibility appeared to be more relevant to differences in physical inactivity across Victorian LGAs than to the other health outcomes examined. The results also suggest that the relationship between recreational accessibility and health may depend on the specific outcome being considered, rather than reflecting a uniform association across all health measures.

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Appendix

Appendix A Additional Model Results (M1 vs M2)

Table 7: Incremental explanatory power of accessibility indicators (M1 → M2). *Note: This is the complete results for all 30 M2 models. For each outcome, the indicator shown is the one associated with the highest adjusted R^2 among the six individual M2 models. Δ Adjusted R^2 is calculated relative to M1.*

Outcome	Indicator added	M2 Adjusted R^2	Δ Adjusted R^2	HC3 p-value
Less than moderate activity	Park distance	0.619	+0.007	0.138
Less than moderate activity	Natural-area distance	0.680	+0.068	0.001
Less than moderate activity	Facility distance	0.621	+0.009	0.162
Less than moderate activity	Indoor-facility distance	0.630	+0.017	0.071
Less than moderate activity	Indoor share (reversed)	0.662	+0.050	0.039
Less than moderate activity	Sport diversity (reversed)	0.609	-0.004	0.701
At least 150 min/week MVPA	Park distance	0.150	-0.011	0.908
At least 150 min/week MVPA	Natural-area distance	0.175	+0.015	0.300
At least 150 min/week MVPA	Facility distance	0.156	-0.005	0.440
At least 150 min/week MVPA	Indoor-facility distance	0.154	-0.007	0.667
At least 150 min/week MVPA	Indoor share (reversed)	0.206	+0.045	0.357
At least 150 min/week MVPA	Sport diversity (reversed)	0.166	+0.005	0.443
Obesity	Park distance	0.641	-0.003	0.630
Obesity	Natural-area distance	0.652	+0.008	0.121
Obesity	Facility distance	0.645	0.000	0.297
Obesity	Indoor-facility distance	0.641	-0.004	0.698
Obesity	Indoor share (reversed)	0.640	-0.004	0.765
Obesity	Sport diversity (reversed)	0.643	-0.001	0.442

Outcome	Indicator added	M2 Adjusted R^2	Δ Adjusted R^2	HC3 p-value
Psychological distress	Park distance	0.315	-0.002	0.347
Psychological distress	Natural-area distance	0.308	-0.009	0.927
Psychological distress	Facility distance	0.322	+0.005	0.152
Psychological distress	Indoor-facility distance	0.319	+0.002	0.355
Psychological distress	Indoor share (reversed)	0.345	+0.028	0.125
Psychological distress	Sport diversity (reversed)	0.308	-0.009	0.973
Fair/poor self-rated health	Park distance	0.465	+0.001	0.301
Fair/poor self-rated health	Natural-area distance	0.470	+0.005	0.165
Fair/poor self-rated health	Facility distance	0.462	-0.002	0.439
Fair/poor self-rated health	Indoor-facility distance	0.458	-0.007	0.915
Fair/poor self-rated health	Indoor share (reversed)	0.464	-0.001	0.297
Fair/poor self-rated health	Sport diversity (reversed)	0.469	+0.005	0.229